



ICD-O-3

Carcinoma, Urothelial NOS
8120/3

Gender: Male

Race: Unknown

Path Site: Bladder (R) Lateral Wall & (L) Anterior Wall
C67.8

Pathology Report:

CSCF: Bladder, Lateral Wall, C67.2

gww 12/24/12

Surgical Pathology Report

FINAL PATHOLOGIC DIAGNOSIS

A. Right pelvic lymph nodes; dissection:

- Eighteen lymph nodes, no tumor (0/18).

B. Left pelvic lymph nodes; dissection:

- Sixteen lymph nodes, no tumor (0/16).

C. Urinary bladder, prostate and seminal vesicles; cystoprostatectomy:

- Bladder with multifocal (X2) invasive urothelial carcinoma, with squamous (40%), microcystic (15%), focal clear cell feature (5%) and focal glandular (5%) differentiation, see pathologic parameters.

- Prostate with prostatic adenocarcinoma, see pathologic parameters.

D. Left distal ureter; excision:

- Portion ureter, no tumor.

E. Right distal ureter; excision:

- Portion ureter, no tumor.

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Invasive urothelial carcinoma, with squamous (40%), microcystic (15%), focal clear cell feature (5%) and focal glandular (5%) differentiation.
2. Grade of tumor: High grade.
3. Depth of invasion: Muscularis propria- outer half.
4. Tumor distribution: Multifocal (X2), 2.2 cm in right lateral wall and 1.7 cm in left anterior wall.
5. Ureteral margins: Negative for tumor.
6. Distal urethral margin: Negative for tumor.
7. Soft tissue margin or serosa: Negative for tumor.
8. Lymph nodes: Negative for tumor (0/34).

See diagnostic
discrepancy form

9. pTNM: pT2b,N0,MX.

Radical Prostatectomy Pathologic Parameters

1. Gleason Score: 3+4=7.
2. Perineural Invasion: Absent.
3. Tumor Location: Peripheral zone - right.
4. Tumor Volume Estimate: 7%.
5. High-grade P.I.N.: Multifocal.
6. Seminal Vesicles: Negative for tumor.
7. Extraprostatic extension: Absent.
8. Peripheral Margin: Negative for tumor.
9. Distal (apical) Margin: Negative for tumor.
10. Proximal (basilar) Margin: Negative for tumor.
11. Regional Lymph Nodes (right and left): Negative for tumor (0/34).
12. pTNM: pT2a,N0,MX.

Effective _____ this Checklist utilizes the 7th edition TNM staging system for Bladder of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

xx

[], MD

Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow, [], MD, PhD

Electronically Signed Out by [] MD

Clinical History:

The patient is a _____ year-old male with bladder cancer undergoing a radical cystectomy with neobladder.

Specimens Received:

- A: Right pelvic lymph nodes
- B: Left pelvic lymph nodes
- C: Bladder, prostate and seminal vesicles
- D: Left distal ureter
- E: Right distal ureter

Gross Description:

The specimens are received in five containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "right pelvic lymph nodes". Received fresh and placed in formalin is an aggregate of fibroadipose

tissue measuring 9.2 x 6.5 x 2.8 cm. Several apparent lymph nodes and lymph node candidates are identified ranging in size from 0.3-3.6 cm in greatest dimension. Representative sections are submitted as follows:

A1-A2: One sectioned lymph node.

A3-A6: One sectioned lymph node, each cassette

A7-A9: 2 lymph nodes, each cassette

A10-A11: 4 lymph node candidates, each cassette

B. The second container is additionally identified as, "left pelvic lymph nodes". Received fresh and placed in formalin is an aggregate of fibroadipose tissue measuring 7.5 x 6.2 x 2.5 cm. Several apparent lymph nodes and lymph node candidates are identified ranging in size from 0.2-3.8 cm in greatest dimension. Representative sections are submitted as follows:

B1-B2: One sectioned lymph node

B3-B4: One sectioned lymph node, each cassette

B5: One lymph node

B6: 2 lymph nodes.

B7-B8: 2 lymph nodes, each cassette

B9-B10: 3 lymph node candidates, each cassette

C. The third container is additionally identified as, "bladder, prostate and seminal vesicles". Received fresh and placed in formalin is a 18.5 x 13.5 x 5.0 cm cystoprostatectomy specimen consisting of a 17.0 x 10.0 x 4.0 cm markedly dilated bladder with diffusely thickened wall and attached mesenteric fat, and a 4.3 x 3.5 x 3.4 cm prostate. The right seminal vesicles measure 3.5 x 1.4 x 0.9 cm and right vas deferens measures 2.5 cm. The left seminal vesicles measures 3.5 x 1.7 x 0.8 cm and left vas deferens measures 3.6 cm. The right ureteral stump measures 0.9 x 0.4 cm, the left measures 0.6 x 0.4 cm, and both demonstrate intact, patent lumens.

The right half of the prostate and bladder is inked blue and the left half is inked black. The bladder and prostate are opened anteriorly along the urethra. The prostate is surrounded by a thin, intact membranous capsule and sectioning demonstrates rubbery, pink-tan, peri-urethral nodularity with no discrete masses or indurations.

The opened bladder reveals a 2.2 x 2.2 cm papillary, friable mass with ulceration located in the right lateral wall. On cut section, the mass extends into but not through the muscularis propria and is 0.9 cm from the deep margin. There is a second papillary, friable mass measuring 1.7 x 1.6 cm, which is located in the left anterior wall. On cut section the mass extends into but not through the muscularis propria and is 1.0 cm from the deep margin. The surrounding bladder mucosa is edematous, granular, pink-tan with a uniform, 1.7 -2.0 cm wall thickness. Bilateral ureteral orifices, adjacent to the trigone, are identified and probe patent.

Representative sections are submitted as follows:

- C1: Distal margin
- C2: Prostate Apex
- C3: Right ureter resection margin
- C4: Left ureter resection margin
- C5-C9: Bladder mass #1, right lateral wall
- C10-C13: Bladder, mass #2, left anterior wall
- C14: Uninvolved bladder mucosa bladder dome
- C15: Uninvolved bladder mucosa anterior wall
- C16: Uninvolved bladder mucosa posterior wall
- C17: Right ureter orifice, trigone
- C18: Left ureter orifice, trigone
- C19-C21: Representative right lobe of prostate from apex to base
- C22-C24: Representative left lobe of prostate from apex to base
- C25: Bilateral seminal vesicles and vas deferens

D. The fourth container is additionally identified as, "left distal ureter".

Received fresh and placed in formalin is a segment of ureter with surrounding periureteral tissues measuring 1.5 cm in length and 1.2 cm in diameter. There is a surgical stitch on one end of the specimen designating margin which is inked blue. The margin is shaved and submitted en face in cassette D1.

E. The fifth container is additionally identified as, "right distal ureter".

Received fresh and placed in formalin is a segment of ureter with surrounding periureteral tissues measuring 1.2 cm in length and 0.7 cm in diameter. There is a surgical stitch on one end of the specimen designating margin which is inked blue. The margin is shaved and submitted en face in cassette E1.

xxx

[] MD, PhD

Criteria	Yes	No
Diagnosis Discrepancy <i>Mixed types > 50%</i>	☒	☐
Primary Tumor Site Discrepancy	☐	☒
HiPAA Discrepancy	☐	☒
Prior Malignancy History	☐	☒
Dual, Synchronous Primary, Noted	☒	☐
Case is (circle) QUALIFIED / DISQUALIFIED	☒	☐
Reviewer Initials <i>LM</i> Date reviewed <i>12/20/12</i>	☒	☐

TCGA Pathologic Diagnosis Discrepancy Form

V4.00

Instructions: The TCGA Pathologic Diagnosis Discrepancy Form should be completed when the pathologic diagnosis documented on the initial pathology report for a case submitted for TCGA is inconsistent with the diagnosis provided on the Case Quality Control Form completed for the submitted case.

Tissue Source Site (TSS): _____ TSS Identifier: _____ TSS Unique Patient Identifier: _____

Completed By (Interviewer Name on OpenClinica) _____ Completed Date: _____

Diagnosis Information			
#	Data Element	Entry Alternatives	Working Instructions
1	Pathologic Diagnosis Provided on Initial Pathology Report	muscle invasive urothelial carcinoma, with squamous (40%), microcystic (15%), focal clear cell features (5%) & focal glandular differentiation	Provide the diagnosis/ histologic subtype(s) documented on the initial pathology report for this case. If the histology for this case is mixed, provide all listed subtypes.
2	Histologic features of the sample provided for TCGA, as reflected on the CQCF.	muscle <u>invasive urothelial carcinoma</u>	Provide the histologic features selected on the TCGA Case Quality Control Form completed for this case.

Discrepancy between Pathology Report and Case Quality Control Form			
3	Provide the reason for the discrepancy between the pathology report and the TCGA Case Quality Control Form.	The tumor is all muscle invasive urothelial carcinoma, high grade, with morphologically variant features.	Provide a reason describing why the diagnosis on the initial pathology report for this case is not consistent with the diagnosis selected on the TCGA Case Quality Control Form.
4	Name of TSS Reviewing Pathologist or Biorepository Director	_____	Provide the name of the pathologist who reviewed this case for TCGA.

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled.

TSS Reviewing Pathologist or Biorepository Director

Date

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled. The Attending Pathologist or the Department Chairman has been informed or is aware of the above discrepancy in diagnoses.

Principal Investigator Signature

Date